You said:

agora apenas traduza esse conteudo para ingles com todas as sessoes e imagens

ChatGPT said:

Ophthalmological Report – English Translation and Medical Explanation

The document is the **report from your ophthalmological consultation on October 29, 2026, in Łódź, with Dr. Ewa Zamojska**, at Centrum Medyczne Avete.

The most important finding is a suspected diagnosis of **right-sided Brown syndrome**. The doctor recommended an MRI of the orbits, followed by an assessment at a specialized strabismus clinic to determine whether further surgery may be appropriate.

I will keep the original sections and structure, including the explanatory illustrations and chart.

1. Document translation

Medical history

Ophthalmological examination record – October 29, 2026

Medical history:

- Previous strabismus surgery performed in Brazil (the document mentions 2012).
- Left eye: recession of the superior rectus muscle by 5.5 mm.

Symptoms:

- Double vision when looking to the left and straight ahead.
- Compensatory head posture: face turned to the left.

Additional information:

- Wears glasses only for reading.
- Denies systemic diseases.
- History of knee surgery in 2016.

Refraction examination

AR – Autorefraction:

Measurement	Right eye	Left eye
Sphere	0.00	+0.25
Cylinder	-1.50	-0.25
Axis	15°	77°

Second measurement (2xT):

Measurement	Right eye	Left eye
Sphere	+0.25	+0.50
Cylinder	-1.25	-0.25
Axis	21°	75°

Visual acuity:

- Right eye: 1.0 (with -1.00); cylindrical correction does not improve vision.
- Left eye: 1.0 without correction.

This corresponds approximately to 20/20 visual acuity in both eyes under the examination conditions.

Ocular motility

Right eye:

- Limitation of elevation when looking straight up: -1.
- Limitation of elevation in adduction (when the eye moves towards the nose): -2.
- Excessive depression in adduction: +1.

Left eye:

- Eye movements within normal limits.

Compensatory head posture:

- Face turned to the left and upwards.

Cover test

Result: right hypotropia.

Hypotropia means that the right eye is vertically deviated downwards relative to the left eye.

Deviation measurements:

The abbreviations L/P indicate the vertical deviation of the left eye relative to the right eye, according to the notation used in the report.

Condition	Recorded result
Distance	0 L/P up to 16
Near	0 L/P 10
Looking upwards	4 L/P 10
Looking downwards	0 L/P 5
Looking right	0 L/P 4
Looking left	0 L/P 20
Head tilt to the right	0 L/P 6
Head tilt to the left	0 L/P 4

The values have been preserved as recorded in the original. The report does not specify the unit of measurement; such deviations are commonly recorded in prism diopters.

TNO (+): positive result on the stereoscopic vision test (binocular depth perception).

Examination of ocular structures

- Anterior segment of both eyes: no abnormalities.
- Optical media are transparent.
- Pupils are symmetrical and round.
- Normal reaction to light and convergence.
- Fundus: optic discs are pink, with well-defined margins.
- Optic disc cupping appropriate for age.
- Retina attached, pink in color.

- Macula without abnormalities.

Medical conclusion

Translation of the final section:

Suspected right-sided Brown syndrome.

An MRI of the orbits is recommended.

Referral to the specialized strabismus treatment clinic at Barlicki Hospital in Łódź is recommended for further investigation and assessment of possible surgical indications.

Page 2 – Instructions for the next appointment

For the next visit, intended for preoperative assessment:

- Apply an ORTOPAD eye patch to the right eye.
- Keep the right eye covered continuously for the six hours preceding the examination.
- The doctor will remove the patch at the clinic during the appointment.

Responsible physician: Dr. Ewa Zamojska.

2. What do these results mean?

The main finding: suspected Brown syndrome



Brown syndrome is a mechanical eye movement disorder, generally associated with the superior oblique muscle tendon and the trochlea, the structure through which this tendon passes.

Its characteristic feature is difficulty elevating the eye when it is turned towards the nose.

This is particularly relevant because the doctor identified exactly this pattern in your right eye:

- Elevation in primary position: -1.
- Elevation in adduction: -2.
- Left eye movement: normal.

The result -2 represents a more pronounced movement limitation.

However, Brown syndrome has not yet been confirmed. The doctor explicitly recorded it as a suspected diagnosis.

3. Why does double vision get worse when looking to the left?

This is one of the most interesting findings in the report.

When you look to the left, your right eye needs to move towards the nose (adduction).

It is precisely in this position that the doctor identified a significant limitation in the elevation of your right eye.

The recorded values illustrate this:

Vertical deviation according to gaze direction

Magnitude recorded in the report; values shown without the zero component of the original notation.

The recorded deviation when looking to the left is five times greater than the deviation when looking to the right.

This helps explain your complaint of diplopia in that direction.

It is also consistent with the need to turn your head to compensate for the eye misalignment.

4. Why did the doctor request an MRI?

An MRI of the orbits (MRI oczodołów) allows the doctor to investigate the structures involved in eye movement.

In your case, it may help assess:

- The superior oblique muscle–tendon–trochlea complex.
- Possible anatomical or inflammatory abnormalities.
- Other causes of restricted eye movements.

The MRI is particularly important because you have already undergone strabismus surgery.

There is an important distinction: your previous surgery involved the left superior rectus muscle, whereas the current suspected Brown syndrome involves the right eye.

This does not necessarily mean that these are two independent problems. The specialist will need to consider your complete surgical history.

5. Why did the doctor instruct you to cover your right eye for six hours?

The instruction on the second page is particularly important.

> **Instructions for the next appointment**
>
> ## 🕐 6 hours
>
> Keep your right eye continuously covered before the examination. Remove the patch only when instructed by the doctor.

The likely purpose is to temporarily interrupt binocular vision, allowing the visual system to stop compensating for some of the eye misalignment.

This way, during the next examination, the doctor may be able to measure the deviation with less influence from the usual compensatory mechanisms.

This procedure may help with surgical planning.

It does not mean that you should regularly use an eye patch as treatment.

6. Does this mean you will need another surgery?

Not necessarily.

The report uses the Polish expression *kwalifikacja do zabiegu*, which means an assessment to determine whether the patient is a suitable candidate for a surgical procedure.

Therefore, the doctor has not yet definitively recommended surgery.

The recommended sequence is:

1. Undergo an MRI of the orbits.
2. Attend the specialized strabismus clinic at Barlicki Hospital in Łódź.
3. Undergo the examination after keeping your right eye covered for six hours.
4. Determine whether surgery is indicated and, if so, which surgical approach would be appropriate.

7. My interpretation of the overall findings

There are four important aspects of this report.

First: your visual acuity is preserved in both eyes. The identified problem is predominantly related to eye alignment and movement, rather than the individual ability of each eye to see.

Second: there is an objective abnormality in the motility of the right eye, particularly when it attempts to look upwards while turned towards the nose.

Third: the deviation varies considerably depending on the direction of gaze. This makes a careful surgical assessment particularly important, because correcting alignment in one position does not automatically guarantee alignment in other positions.

Fourth: the positive TNO test suggests that you still have some stereoscopic vision, a potentially favorable finding, although it cannot independently predict the outcome of treatment.

One detail that should be clarified

The report records your previous surgery as having been performed in 2012. However, your previous surgical history indicates August 13, 2014, with a 5.5 mm recession of the left superior rectus muscle.

I recommend requesting a correction of this information in your medical records before any surgical planning.

My conclusion: the consultation provided a specific diagnostic hypothesis that may explain your double vision, but confirmation of Brown syndrome and the decision regarding possible surgery depend on further investigation. The report does not state that your previous surgery caused the current problem.